

GATE A PROTECTED REVIEW - CareBridge Clinics

RC2 | Recomputed quality/metrics | Evidence-chain diagnosis | State machine unchanged

Open only after a genuine saved Gate attempt. Diagnose the first attempt; never use this as a pre-attempt solution.

Layer 1 - Data-quality controls

Control	Expected
Valid rows	258
Rejected rows	3
Rejected CB0082	Missing Clinic
Rejected CB0145	Invalid negative Wait_Minutes
Rejected duplicate	CB0016 exact duplicate ID row

Layer 2 - Core protected metrics

Metric / slice	Expected
Overall no-show rate	15.1163%
Overall avg wait	36.89 min
Overall avg satisfaction	3.58
Riverside	n=76; no-show 7.8947%; wait 50.71; sat 3.28
NorthCare	n=65; no-show 29.2308%; wait 31.08; sat 3.65
HillView	n=51; no-show 13.7255%; wait 31.94; sat 3.73
CentralCare	n=66; no-show 10.6061%; wait 30.53; sat 3.74

Layer 3 - Deeper diagnostic controls

Slice	Expected
Phone	n=112; no-show 21.43%; wait 36.14
WalkIn	n=52; no-show 9.62%; wait 42.15
App	n=94; no-show 10.64%; wait 34.87
New vs FollowUp no-show	15.29% vs 15.03% - nearly the same

What a strong finding should say

The two most decision-relevant clinic patterns differ: NorthCare has the highest no-show rate, while Riverside has the longest wait and lowest satisfaction. Do not collapse them into one "worst clinic" without stating which KPI drives the decision.

Causal boundary

Wait/no-show association is not causal proof. Booking channel, patient mix, clinic operations, scheduling policy or selection may confound the relationship.

Evidence-chain pass rubric

Layer	Pass condition
Decision frame	Names the decision, analytical question, scope/comparison and Definition of Done
KPI contract	Numerator/denominator or formula, exclusions, grain and owner are explicit
Quality	Rejected rows are visible; no silent cleaning
Diagnosis	Prioritized comparisons; weighted/reconciled where required; one independent check
Findings	Three findings each include comparison, magnitude, scope and implication
Causal boundary	Observation/association separated from mechanism; alternatives named
Recommendation	Owner + action + timing + success measure + risk/dependency + stop/continue rule
Communication	Decision-first memo and 3-slide message-headline story
Defense	Answers denominator, causal and missing-data challenges without changing definitions

Targeted repair routing

Failure pattern	Repair focus
Decision unclear / analysis starts with charts	IBA1
Metric/grain/denominator drift	IBA2
Too many low-value cuts / no priority	IBA3-IBA4
Causal overclaim	IBA5
Finding is only a number or chart label	IBA6
Generic recommendation	IBA7
Memo buries decision	IBA8
Decorative or topic-title slides	IBA9
Defense changes definitions or hides uncertainty	IBA10
Broken end-to-end traceability	IBA11

Before a fresh retest

- Keep the original failed attempt.
- Write the exact failed competency and repair evidence.
- Close this review before changed evidence.
- A polished story cannot compensate for wrong metrics or unsupported causal claims.

Fresh retry and state route

Clean Gate A pass ends M09. Failed A -> saved attempt -> Review A -> targeted repair -> changed evidence. Gate B only after explicit assignment.

Repair only the named weak competency; do not retake by copying the same control table.